



Tests you can trust

Name : Shivek Sharma(24Y/M)

Date : 23 Mar, 2026

Test Asked : Aarogyam C Pro including Crm With Utsh

Report Status : Complete Report



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First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]

Patient Name : SHIVEK SHARMA (24Y/M)
Referred By : SELF
Home Collection : FROM Nandini Metro Suites Sector 4 Vaishali, Ghaziabad - 201010

Tests Done : AAROGRAM C PRO INCLUDING CRM
WITH UTSH

Report Availability Summary

Note: Please refer to the table below for status of your tests.

 **14** Ready  **0** Ready with Cancellation  **0** Processing  **0** Cancelled in Lab

TEST DETAILS

REPORT STATUS

AAROGRAM C PRO INCLUDING CRM WITH UTSH	Ready 
CHLORIDE	Ready 
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	Ready 
Lipoprotein (a) [Lp(a)]	Ready 
SODIUM	Ready 
TESTOSTERONE	Ready 
HBA PROFILE	Ready 
HEMOGRAM - 6 PART (DIFF)	Ready 
LIVER FUNCTION TESTS	Ready 
IRON DEFICIENCY PROFILE	Ready 
KIDPRO	Ready 
LIPID PROFILE	Ready 
T3-T4-USTSH	Ready 
VITAMIN D TOTAL AND B12 COMBO	Ready 
APOLIPROTEIN RATIO	Ready 

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Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
HEMATOCRIT(PCV)	39.3	%	40.0-50.0
HEMOGLOBIN	12.3	g/dL	13.0-17.0
LYMPHOCYTE	42.2	%	20-40
MEAN CORP.HEMO.CONC(MCHC)	31.3	g/dL	31.5-34.5
MEAN PLATELET VOLUME(MPV)	12.2	fL	6.5-12
PLATELET TO LARGE CELL RATIO(PLCR)	42.5	%	19.7-42.4
TOTAL RBC	4.43	X 10 ⁶ /μL	4.5-5.5
LIPID			
HDL / LDL RATIO	0.4	Ratio	> 0.40
LDL CHOLESTEROL - DIRECT	109	mg/dL	< 100
RENAL			
URIC ACID	7.68	mg/dL	4.2 - 7.3

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.



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Sample Collected on (SCT) : 23 Mar 2026 08:00
Sample Received on (SRT) : 23 Mar 2026 14:44
Report Released on (RRT) : 23 Mar 2026 17:52
Sample Type | Barcode : SERUM | EQ772837

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	E.C.L.I.A	68	ng/mL
Bio. Ref. Interval. : Deficiency : <=20 ng/ml Insufficiency : 21-29 ng/ml Sufficiency : >= 30 ng/ml Toxicity : >100 ng/ml Clinical Significance: Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health. Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome. Increase in vitamin D total levels indicate Vitamin D intoxication. Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50% Kit Validation Reference : Holick M. Vitamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002;9(1)87-98. Method : Fully Automated Electrochemiluminescence Competitive Immunoassay			
VITAMIN B-12	E.C.L.I.A	251	pg/mL
Bio. Ref. Interval. : Normal: 197-771 pg/ml Clinical significance : Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings. Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 % Kit Validation Reference : Thomas L.Clinical laboratory Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431 Method : Fully Automated Electrochemiluminescence Competitive Immunoassay			
Please correlate with clinical conditions.			

Tests Done : AAROYAM C PRO INCLUDING CRM WITH UTSH

Report Remarks : Labcode:2303085999/NCR01

Sneha
Dr.Sneha Singh
MD(Path)



Scan QR to verify(valid for 30 days from release time)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - A1 (APO-A1) Bio. Ref. Interval. : Male : 86 - 152 Female : 94 - 162 Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY - BECKMAN COULTER	IMMUNOTURBIDIMETRY	112	mg/dL
APOLIPOPROTEIN - B (APO-B) Bio. Ref. Interval. : Male : 56 - 145 Female : 53 - 138 Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY - BECKMAN COULTER	IMMUNOTURBIDIMETRY	83	mg/dL
APO B / APO A1 RATIO (APO B/A1) Bio. Ref. Interval. : Male : 0.40 - 1.26 Female : 0.38 - 1.14 Clinical Significance : • Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol. • Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD. • The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc. • An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk. • All results have to be interpreted in Conjunction with clinical history and other findings. Method : Derived from serum Apo A1 and Apo B values	CALCULATED	0.7	Ratio

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
LIPOPROTEIN (A) [LP(A)] Bio. Ref. Interval. :-	IMMUNOTURBIDIMETRY	29.3	mg/dL

Adults : < 30.0 mg/dl

Clinical Significance:

Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:

Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:

Tietz NW, Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

Please correlate with clinical conditions.

Method:- LATEX ENHANCED IMMUNOTURBIDIMETRY

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TEST NAME	TECHNOLOGY	VALUE	UNITS
TESTOSTERONE Bio. Ref. Interval. :- 280 - 800	E.C.L.I.A	631	ng/dL

Clinical Significance: Clinical evaluation of serum testosterone, along with serum LH, assists in evaluation of Hypogonadal males. Major causes of lowered testosterone in males include Hypogonadotropic hypogonadism, testicular failure Hyperprolactinemia, Hypopituitarism some types of liver and kidney diseases and critical illness.

Specifications: Precision: Intra assay (%CV): 11.50 %, Inter assay (%CV): 5.70%; Sensitivity: 7 ng/dL.
Kit Validation Reference: Wilson JD Foster DW (Eds) Williams Textbook of Endocrinology 8th Edition WB Saunders Philadelphia Pennsylvania.

Note : The Biological Reference Range mentioned is specific to the age group and gender. Kindly correlate clinically.

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Competitive Immunoassay

Tests Done : AAROGYAM C PRO INCLUDING CRM WITH UTSH

Report Remarks : Labcode:2303085999/NCR01

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP) Bio. Ref. Interval. :-	IMMUNOTURBIDIMETRY	0.43	mg/L

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection, active arthritis or concurrent illness.

Clinical significance:

High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:

- 1.Clinical management of laboratory data in medical practice 2003-3004, 207(2003).
- 2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

Please correlate with clinical conditions.

Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 Method : Ferrozine method without deproteinization	PHOTOMETRY	105.76	µg/dL
TOTAL IRON BINDING CAPACITY (TIBC) Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl Method : Spectrophotometric Assay	PHOTOMETRY	352.6	µg/dL
% TRANSFERRIN SATURATION Bio. Ref. Interval. : 13 - 45 Method : Derived from IRON and TIBC values	CALCULATED	29.99	%
UNSAT. IRON-BINDING CAPACITY (UIBC) Bio. Ref. Interval. : 162 - 368 Method : SPECTROPHOTOMETRIC ASSAY	PHOTOMETRY	246.84	µg/dL

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	175	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	43	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	109	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	128	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	4.1	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	2.98	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	2.5	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.4	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	132.14	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	25.67	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDH - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	80.66	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.67	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.12	mg/dL	0 - 0.20
BILIRUBIN (INDIRECT)	CALCULATED	0.55	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	11.09	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	22.9	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	14.9	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	1.54	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.63	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.49	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.14	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.43	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Diazonium salt DPD method
BILD - Diazonium salt DPD method
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	11.7	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.77	mg/dL	0.72-1.18
BUN / SR.CREATININE RATIO	CALCULATED	15.2	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	25.04	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	32.52	Ratio	< 52
CALCIUM	PHOTOMETRY	9.99	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	7.68	mg/dL	4.2 - 7.3
SODIUM	I.S.E - INDIRECT	139.9	mmol/L	136 - 145
CHLORIDE	I.S.E - INDIRECT	103.7	mmol/L	98 - 107

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method
SOD - ION SELECTIVE ELECTRODE - INDIRECT
CHL - ION SELECTIVE ELECTRODE - INDIRECT

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	117	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	8.54	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	2.64	µIU/mL	0.54-5.30

Comments : ***

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4 - Fully Automated Electrochemiluminescence Competitive Immunoassay

USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR) Bio. Ref. Interval. :-	CALCULATED	128	mL/min/1.73 m2

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

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Sample Collected on (SCT) : 23 Mar 2026 08:00
Sample Received on (SRT) : 23 Mar 2026 14:46
Report Released on (RRT) : 23 Mar 2026 17:03
Sample Type | Barcode : EDTA Whole Blood | EZ595775

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	4.8	%

Bio. Ref. Interval. :

As per ADA Guidelines

Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
≥6.5% : Diabetic

Guidance For Known Diabetics

Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
≥8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG) CALCULATED 91 mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HbA1c values

Please correlate with clinical conditions.

Tests Done : HBA PROFILE,HEMOGRAM

Report Remarks : Labcode:2303086189/NCR01



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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	12.3	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	39.3	%	40.0-50.0
Total RBC	HF & EI	4.43	X 10⁶/μL	4.5-5.5
Mean Corpuscular Volume (MCV)	Calculated	88.7	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	27.8	pg	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	31.3	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	41.3	fL	39-46
Red Cell Distribution Width (RDW - CV)	Calculated	12.7	%	11.6-14
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	254.3	-	*Refer Note below
MENTZER INDEX	Calculated	20	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	6.83	X 10³ / μL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	52	%	40-80
Lymphocytes Percentage	Flow Cytometry	42.2	%	20-40
Monocytes Percentage	Flow Cytometry	3.8	%	2-10
Eosinophils Percentage	Flow Cytometry	1.8	%	1-6
Basophils Percentage	Flow Cytometry	0.3	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	3.55	X 10 ³ / μL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.88	X 10 ³ / μL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.26	X 10 ³ / μL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.02	X 10 ³ / μL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.12	X 10 ³ / μL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10 ³ / μL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10 ³ / μL	0.0-0.5
PLATELET COUNT	HF & EI	204	X 10³ / μL	150-410
Mean Platelet Volume (MPV)	Calculated	12.2	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	15.2	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	42.5	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.25	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

~~ End of report ~~

Tests Done : HBA PROFILE,HEMOGRAM

Report Remarks : Labcode:2303086189/NCR01

Sneha

Dr.Sneha Singh
MD(Path)



Scan QR to verify(valid for 30 days from release time)

CUSTOMER DETAILS

As declared in our data base

Name: SHIVEK SHARMA **Age:** 24Y **Sex:** M

Barcodes/Sample_Type : EQ772837(SERUM), EZ595775(EDTA)
Labcode : 2303085999, 2303086189
Ref By : SELF
Sample_Type/Tests : SERUM:AAROGYAM C PRO INCLUDING CRM WITH UTSH
EDTA:HBA PROFILE, HEMOGRAM - 6 PART (DIFF)
Sample Collected At : FROM Nandini Metro Suites Sector 4 Vaishali, Ghaziabad - 201010
Sample Collected on (SCT) : 23 Mar 2026 08:00
Report Released on (RRT) : 23 Mar 2026 17:52
Amount Collected : -

Thyrocare,D-37/1,MIDC,Turbhe,Navi Mumbai - 400703. | Phone:022 - 6712 3400 | www.thyrocare.com | info@thyrocare.com

CONDITIONS OF REPORTING

- ✓ The reported results are for information and interpretation of the referring doctor only.
- ✓ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ✓ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ✓ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ✓ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ✓ This report is not valid for medico-legal purpose.
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EXPLANATIONS

- ✓ Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- ✓ **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- ✓ **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- ✓ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- ✓ **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- ✓ **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- ✓ **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- ✓ **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- ✓ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ✓ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- ✓ Values out of reference range requires reconfirmation before starting any medical treatment.
- ✓ Retesting is needed if you suspect any quality shortcomings.
- ✓ Testing or retesting should be done in accredited laboratories.
- ✓ For suggestions, complaints, clinical support or feedback, write to us at customersupport@thyrocare.com or call us on **022-3090 0000**

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* T&C Apply, #As on 5th December 2024 (Applicable for all company owned labs except Bhagalpur & Vijayawada),

* As per survey on doctors' perception of laboratory diagnostics (IJARIIT, 2023), -Mumbai Reference Lab is CAP Accredited